

A. Passively allows others to assume responsibility for major areas of life because of inability to function independently (e.g., lets spouse decide what kind of job he or she should have).

B. Subordinates own needs to those of persons on whom he or she depends in order to avoid any possibility of having to rely on self, e.g., tolerates abusive spouse.

C. Lacks self-confidence, e.g., sees self as helpless, stupid.

301.40 Compulsive Personality Disorder

The essential feature is a Personality Disorder (p. 305) in which there generally are restricted ability to express warm and tender emotions; perfectionism that interferes with the ability to grasp "the big picture"; insistence that others submit to his or her way of doing things; excessive devotion to work and productivity to the exclusion of pleasure; and indecisiveness.

Individuals with this disorder are stingy with their emotions and material possessions. For example, they rarely give compliments or gifts. Everyday relationships have a conventional, formal, and serious quality. Others often perceive these individuals as stilted and "stiff."

Preoccupation with rules, efficiency, trivial details, procedures, or form interferes with the ability to take a broad view of things. For example, such an individual, having misplaced a list of things to be done, will spend an inordinate amount of time looking for the list rather than spend a few moments to recreate the list from memory and proceed with accomplishing the activities. Time is poorly allocated, the most important tasks being left to the last moment. Although efficiency and perfection are idealized, they are rarely attained.

Individuals with this disorder are always mindful of their relative status in dominance-submission relationships. Although they resist the authority of others, they stubbornly insist that people conform to their way of doing things. They are unaware of the feelings of resentment or hurt that this behavior evokes in others. For example, a husband may insist that his wife complete errands for him regardless of her plans.

Work and productivity are prized to the exclusion of pleasure and the value of interpersonal relationships. When pleasure is considered, it is something to be planned and worked for. However, the individual usually keeps postponing the pleasurable activity, such as a vacation, so that it may never occur.

Decision-making is avoided, postponed, or protracted, perhaps because of an inordinate fear of making a mistake. For example, assignments cannot be completed on time because the individual is ruminating about priorities.

Associated features. Individuals with this disorder may complain of difficulty expressing tender feelings. Considerable distress is often associated with their indecisiveness and general ineffectiveness. Their speech may be circumstantial. Depressed mood is common. Individuals with this disorder tend to be

excessively conscientious, moralistic, scrupulous, and judgmental of self and others. (For example, a man believed it was "sinful" for his neighbor to leave his child's bicycle in the rain.) When they are unable to control others, a situation, or their environment, they often ruminate about the situation and become angry, although the anger is usually not expressed directly. (For example, a man may be angry when service in a restaurant is poor, but instead of complaining to the management, ruminates about how much he will leave as a tip.) Frequently there is extreme sensitivity to social criticism, especially if it comes from someone with considerable status or authority.

Impairment. This disorder frequently is quite incapacitating, particularly in its effect on occupational functioning.

Complications. Obsessive Compulsive Disorder, Hypochondriasis, Major Depression, and Dysthymic Disorder may be complications. Many of the features of Compulsive Personality Disorder are apparently present in individuals who develop myocardial infarction.

Predisposing factors. No information.

Prevalence and sex ratio. The disorder is apparently common and is more frequently diagnosed in men.

Familial pattern. The disorder is apparently more common among family members than in the general population.

Differential diagnosis. In **Obsessive Compulsive Disorder** there are, by definition, true obsessions and compulsions, which are not present in Compulsive Personality Disorder. However, if the criteria for both disorders are met, both diagnoses should be recorded.

Diagnostic criteria for Compulsive Personality Disorder

At least four of the following are characteristic of the individual's current and long-term functioning, are not limited to episodes of illness, and cause either significant impairment in social or occupational functioning or subjective distress.

- (1) restricted ability to express warm and tender emotions, e.g., the individual is unduly conventional, serious and formal, and stingy
- (2) perfectionism that interferes with the ability to grasp "the big picture," e.g., preoccupation with trivial details, rules, order, organization, schedules, and lists
- (3) insistence that others submit to his or her way of doing things, and lack of awareness of the feelings elicited by this behavior, e.g., a husband stubbornly insists his wife complete errands for him regardless of her plans

(4) excessive devotion to work and productivity to the exclusion of pleasure and the value of interpersonal relationships

(5) indecisiveness: decision-making is either avoided, postponed, or protracted, perhaps because of an inordinate fear of making a mistake, e.g., the individual cannot get assignments done on time because of ruminating about priorities

301.84 Passive-Aggressive Personality Disorder

The essential feature is a Personality Disorder (p. 305) in which there is resistance to demands for adequate performance in both occupational and social functioning; the resistance is expressed indirectly rather than directly. The consequence is pervasive and persistent social or occupational ineffectiveness, even when more self-assertive and effective behavior is possible. The name of this disorder is based on the assumption that such individuals are passively expressing covert aggression.

Individuals with this disorder habitually resent and oppose demands to increase or maintain a given level of functioning. This occurs most clearly in work situations, but is also evident in social functioning. The resistance is expressed indirectly, through such maneuvers as procrastination, dawdling, stubbornness, intentional inefficiency, and "forgetfulness." For example, when an executive gives a subordinate some material to review for a meeting the next morning, rather than complain that he or she has no time to do the work, the subordinate may misplace or misfile the material and thus attain his or her goal by passively resisting the demand on him or her. Similarly, when an individual always comes late to appointments, promises to help make arrangements for particular events but never does, and keeps "forgetting" to bring important documents to club meetings, he or she is passively resisting demands made on him or her by others.

The individual is ineffective both socially and occupationally because of the passive-resistant behavior. For example, job promotions are not offered because of the individual's intentional inefficiency. A housewife with the disorder fails to do the laundry or to stock the kitchen with food because of procrastination and dawdling.

For the diagnosis to be made, it is essential that this pattern of behavior occur in a variety of contexts in which more adaptive functioning is clearly possible.

Associated features. Often individuals with this disorder are dependent and lack self-confidence. Typically, they are pessimistic about the future but have no realization that their behavior is responsible for their difficulties. Although the individual may experience conscious resentment against authority figures, he or she never connects his or her passive-resistant behavior with this resentment.

Impairment. By definition, some impairment in social and occupational functioning is always present.

Complications. Frequent complications include Major Depression, Dysthymic Disorder, and Alcohol Abuse or Dependence.

Predisposing factors. Oppositional Disorder in childhood or adolescence apparently predisposes to the development of this disorder.

Prevalence, sex ratio, and familial pattern. No information.

Differential diagnosis. In **Oppositional Disorder** there may be a similar clinical picture and this diagnosis preempts the diagnosis of Passive-Aggressive Personality Disorder if the individual is under 18.

Passive-aggressive maneuvers that are used in certain situations in which assertive behavior is discouraged or actually punished and that are not part of a pervasive pattern of personality functioning do not warrant this diagnosis.

Diagnostic criteria for Passive-Aggressive Personality Disorder

The following are characteristic of the individual's current and long-term functioning, and are not limited to episodes of illness.

A. Resistance to demands for adequate performance in both occupational and social functioning.

B. Resistance expressed indirectly through at least two of the following:

- (1) procrastination
- (2) dawdling
- (3) stubbornness
- (4) intentional inefficiency
- (5) "forgetfulness"

C. As a consequence of A and B, pervasive and long-standing social and occupational ineffectiveness (including in roles of housewife or student), e.g., intentional inefficiency that has prevented job promotion.

D. Persistence of the behavior pattern even under circumstances in which more self-assertive and effective behavior is possible.

E. Does not meet the criteria for any other Personality Disorder, and if under age 18, does not meet the criteria for Oppositional Disorder.

301.89 Atypical, Mixed, or Other Personality Disorder

If an individual qualifies for any of the specific Personality Disorders, that category should be noted even if some features from other categories are present. For example, an individual who fits the description of Compulsive Personality Disorder should be given that diagnosis even if some mild dependent or paranoid features are present.

When an individual qualifies for two or more Personality Disorders, multiple diagnoses should be made.

Atypical Personality Disorder should be used when the clinician judges that a Personality Disorder is present but there is insufficient information to make a more specific designation.

Mixed Personality Disorder should be used when the individual has a Personality Disorder that involves features from several of the specific Personality Disorders, but does not meet the criteria for any one Personality Disorder.

Other Personality Disorder should be used when the clinician judges that a specific Personality Disorder not included in this classification is appropriate, such as Masochistic, Impulsive, or Immature Personality Disorder. In such instances the clinician should record the specific Other Personality Disorder, using the 301.89 code.

V Codes For Conditions Not Attributable To a Mental Disorder That Are A Focus of Attention Or Treatment

The ICD-9-CM includes V Codes for a "Supplementary Classification of Factors Influencing Health Status and Contact with Health Services." A brief list of V Codes adapted from ICD-9-CM is provided here for conditions that are a focus of attention or treatment but are not attributable to any of the mental disorders noted previously. In some instances one of these conditions will be noted because, after a thorough evaluation, no mental disorder is found. In other instances the scope of the diagnostic evaluation has not been adequate to determine the presence or absence of a mental disorder but there is a need to note the reason for contact with the mental health care system. (With further information, the presence of a mental disorder may become apparent.) Finally, an individual may have a mental disorder, but the focus of attention or treatment is on a condition that is not due to the mental disorder. For example, an individual with Bipolar Disorder may have marital problems that are not directly related to manifestations of the Affective Disorder but are the principal focus of treatment.

V65.20 Malingering

The essential feature is the voluntary production and presentation of false or grossly exaggerated physical or psychological symptoms. The symptoms are produced in pursuit of a goal that is obviously recognizable with an understanding of the individual's circumstances rather than of his or her individual psychology. Examples of such obviously understandable goals include: to avoid military conscription or duty, to avoid work, to obtain financial compensation, to evade criminal prosecution, or to obtain drugs.

Under some circumstances Malingering may represent adaptive behavior, for example, feigning illness while a captive of the enemy during wartime.

A high index of suspicion of Malingering should be aroused if any combination of the following is noted:

- (1) medicolegal context of presentation, e.g., the person's being referred by his attorney to the physician for examination;
- (2) marked discrepancy between the person's claimed distress or disability and the objective findings;
- (3) lack of cooperation with the diagnostic evaluation and prescribed treatment regimen;
- (4) the presence of Antisocial Personality Disorder.

The differentiation of Malingering from Factitious Disorder depends on the clinician's judgment as to whether the symptom production is in pursuit of a goal that is obviously recognizable and understandable in the circumstances. Individuals with Factitious Disorders have goals that are not recognizable in

light of their specific circumstances but are understandable only in light of their psychology as determined by careful examination. Evidence of an intrapsychic need to maintain the sick role suggests Factitious Disorder. Thus, the diagnosis of Factitious Disorder excludes the diagnosis of the act of Malingering.

Malingering is differentiated from Conversion and the other Somatoform Disorders by the voluntary production of symptoms and by the obvious, recognizable goal. The malingering individual is much less likely to present his or her symptoms in the context of emotional conflict, and the symptoms presented are less likely to be "symbolic" of an underlying emotional conflict. Symptom relief in Malingering is not often obtained by suggestion, hypnosis, or intravenous barbiturates, as it frequently is in Conversion Disorder.

V62.89 Borderline Intellectual Functioning

This category can be used when a focus of attention or treatment is associated with Borderline Intellectual Functioning, i.e., an IQ in the 71-84 range. The differential diagnosis between Borderline Intellectual Functioning and Mental Retardation (an IQ of 70 or below) is especially difficult and important when certain mental disorders coexist. For example, when the diagnosis is of Schizophrenic Disorder, Undifferentiated or Residual Type, and impairment in adaptive functioning is prominent, the existence of Borderline Intellectual Functioning is easily overlooked, and hence the level and quality of potential adaptive functioning may be incorrectly assessed.

V71.01 Adult Antisocial Behavior

This category can be used when a focus of attention or treatment is adult antisocial behavior that is apparently not due to a mental disorder, such as a Conduct Disorder, Antisocial Personality Disorder, or a Disorder of Impulse Control. Examples include the behavior of some professional thieves, racketeers, or dealers in illegal substances.

V71.02 Childhood or Adolescent Antisocial Behavior

Same as above. Examples include isolated antisocial acts of children or adolescents (not a pattern of antisocial behavior).

V62.30 Academic Problem

This category can be used when a focus of attention or treatment is an academic problem that is apparently not due to a mental disorder. An example is a pattern of failing grades or of significant underachievement in an individual with adequate intellectual capacity, in the absence of a Specific Developmental Disorder or any other mental disorder to account for the problem.

V62.20 Occupational Problem

This category can be used when a focus of attention or treatment is an occupational problem that is apparently not due to a mental disorder. Examples include job dissatisfaction and uncertainty about career choices.

V62.82 Uncomplicated Bereavement

This category can be used when a focus of attention or treatment is a normal reaction to the death of a loved one (bereavement).

A full depressive syndrome frequently is a normal reaction to such a loss, with feelings of depression and such associated symptoms as poor appetite, weight loss, and insomnia. However, morbid preoccupation with worthlessness, prolonged and marked functional impairment, and marked psychomotor retardation are uncommon and suggest that the bereavement is complicated by the development of a Major Depression.

In Uncomplicated Bereavement, guilt, if present, is chiefly about things done or not done at the time of the death by the survivor; thoughts of death are usually limited to the individual's thinking that he or she would be better off dead or that he or she should have died with the person who died. The individual with Uncomplicated Bereavement generally regards the feeling of depressed mood as "normal," although he or she may seek professional help for relief of such associated symptoms as insomnia and anorexia.

The reaction to the loss may not be immediate, but rarely occurs after the first two or three months. The duration of "normal" bereavement varies considerably among different subcultural groups.

V15.81 Noncompliance with Medical Treatment

This category can be used when a focus of attention or treatment is noncompliance with medical treatment that is apparently not due to a mental disorder. Examples include failure to follow a prescribed diet because of religious beliefs or to take required medication because of a considered decision that the treatment is worse than the illness. The major differential is with Personality Disorders with prominent paranoid, passive-aggressive, or masochistic features.

V62.89 Phase of Life Problem or Other Life Circumstance Problem

This category can be used when a focus of attention or treatment is a problem associated with a particular developmental phase or some other life circumstance that is apparently not due to a mental disorder. Examples include problems associated with going to school, separating from parental control, starting a new career, marriage, divorce, and retirement.

V61.10 Marital Problem

This category can be used when a focus of attention or treatment is a marital problem that is apparently not due to a mental disorder. An example is marital conflict related to estrangement or divorce.

V61.20 Parent-Child Problem

This category can be used when a focus of attention or treatment is a parent-child problem that is apparently not due to a mental disorder of the individual (parent or child) who is being evaluated. An example is child abuse not attributable to a mental disorder of the parent.

V61.80 Other Specified Family Circumstances

This category can be used when a focus of attention or treatment is a family

circumstance that is apparently not due to a mental disorder and is not a Parent-Child or a Marital Problem. Examples are interpersonal difficulties with an aged in-law, or sibling rivalry.

V62.81 Other Interpersonal Problem

This category can be used when a focus of attention or treatment is an interpersonal problem (other than marital or parent-child) that is apparently not due to a mental disorder of the individual who is being evaluated. Examples are difficulties with co-workers, or with romantic partners.

Additional Codes*

300.90 Unspecified Mental Disorder (nonpsychotic)

This is a residual category to be used when enough information is available to rule out a psychotic disorder, but further specification is not possible. In some cases, with more information, the diagnosis can be changed to a specific disorder.

V71.09 No Diagnosis or Condition on Axis I

This category should be used to indicate that following an examination, no Axis I diagnosis or condition (including the V code categories) is present. There may or may not be an Axis II diagnosis.

799.90 Diagnosis or Condition Deferred on Axis I

This category should be used to indicate that there is insufficient information to make any diagnostic judgment about an Axis I diagnosis or condition.

V71.09 No Diagnosis on Axis II

This category should be used to indicate that, following an examination, no Axis II diagnosis (i.e., no Personality Disorder or Specific Developmental Disorder) is present. There may or may not be an Axis I diagnosis or condition.

799.90 Diagnosis Deferred on Axis II

This category should be used to indicate that there is insufficient information to make any diagnostic judgment about an Axis II diagnosis.

* Although the terms distinguish between Axis I and Axis II, in order to maintain compatibility with ICD-9-CM, these Axis I and Axis II codes are the same.

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Decision Trees For Differential Diagnosis

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Appendix A:

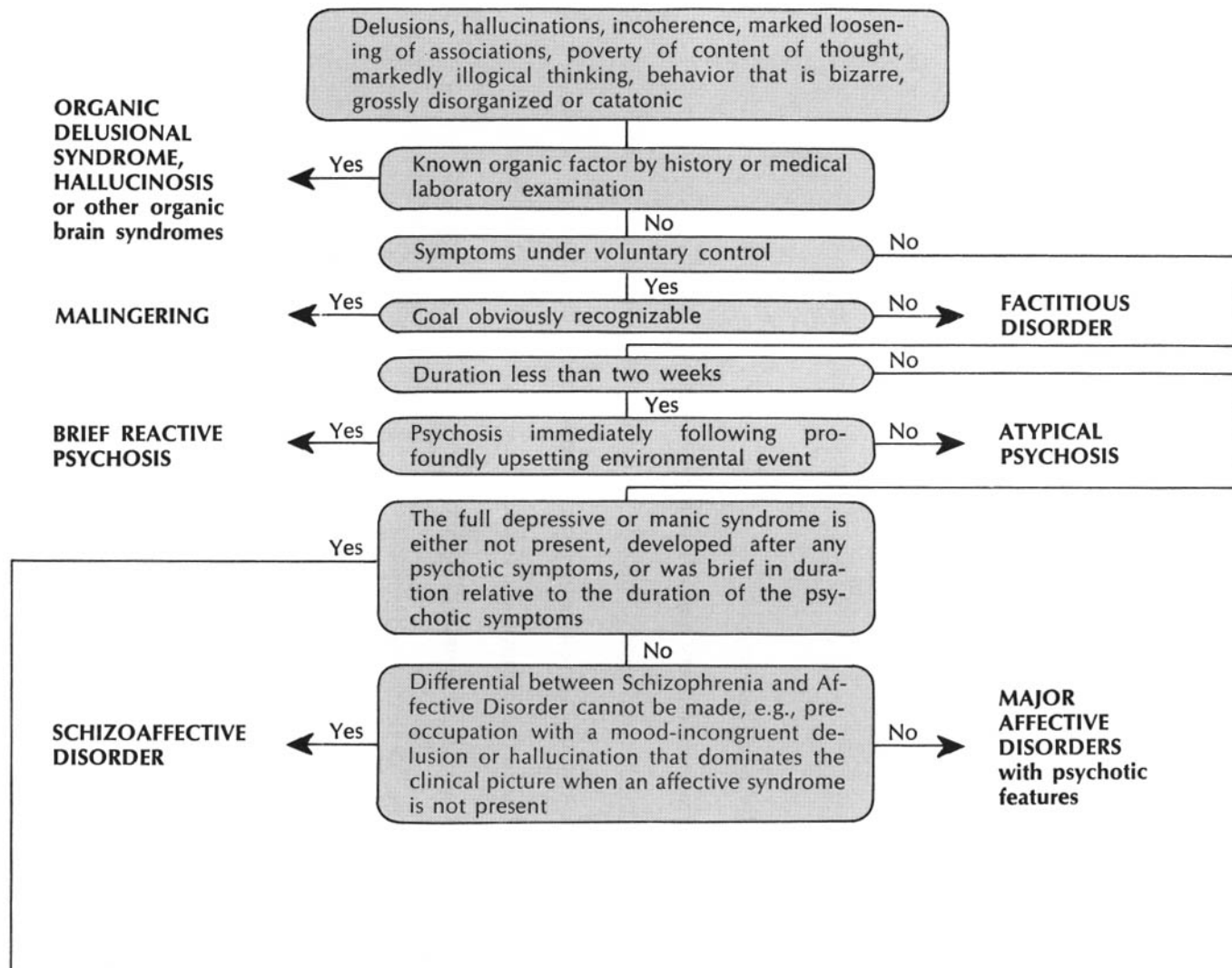
Decision Trees For Differential Diagnosis*

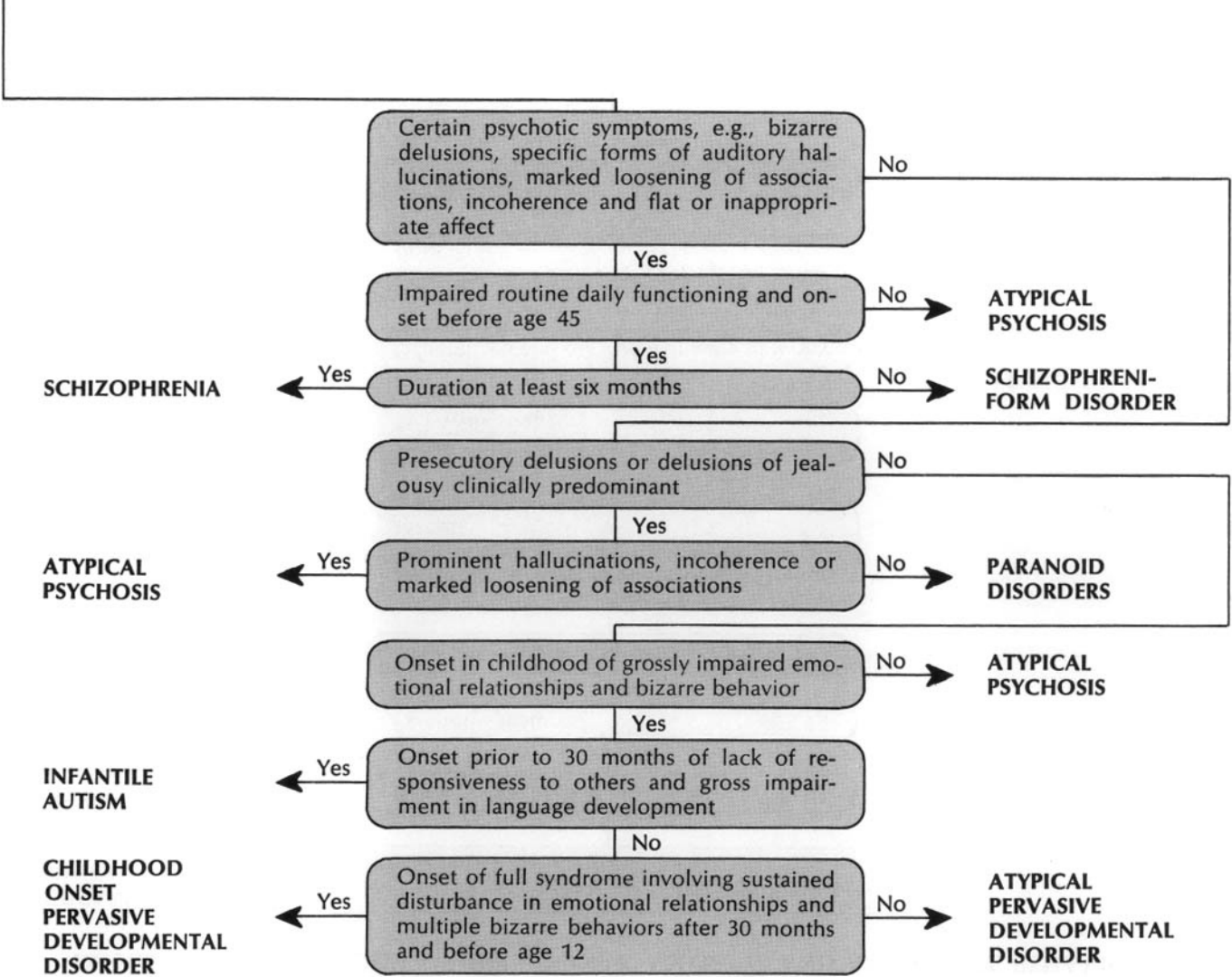
The purpose of these decision trees is to aid the clinician in understanding the organization and hierarchical structure of the classification. Each decision tree starts with a set of clinical features. When one of these features is a prominent part of the presenting clinical picture, the clinician can follow the series of questions to rule in or out various diagnostic categories. The questions are only approximations of the actual diagnostic criteria. The decision trees are not meant to replace the specific diagnostic criteria.

	Page
Psychotic features	340
Irrational anxiety and avoidance behavior	342
Mood disturbance (depressed, irritable or expansive)	344
Antisocial, aggressive, defiant, or oppositional behavior	345
Physical complaints and irrational anxiety about physical illness	346
Academic or learning difficulties	348
Organic Brain Syndromes	349

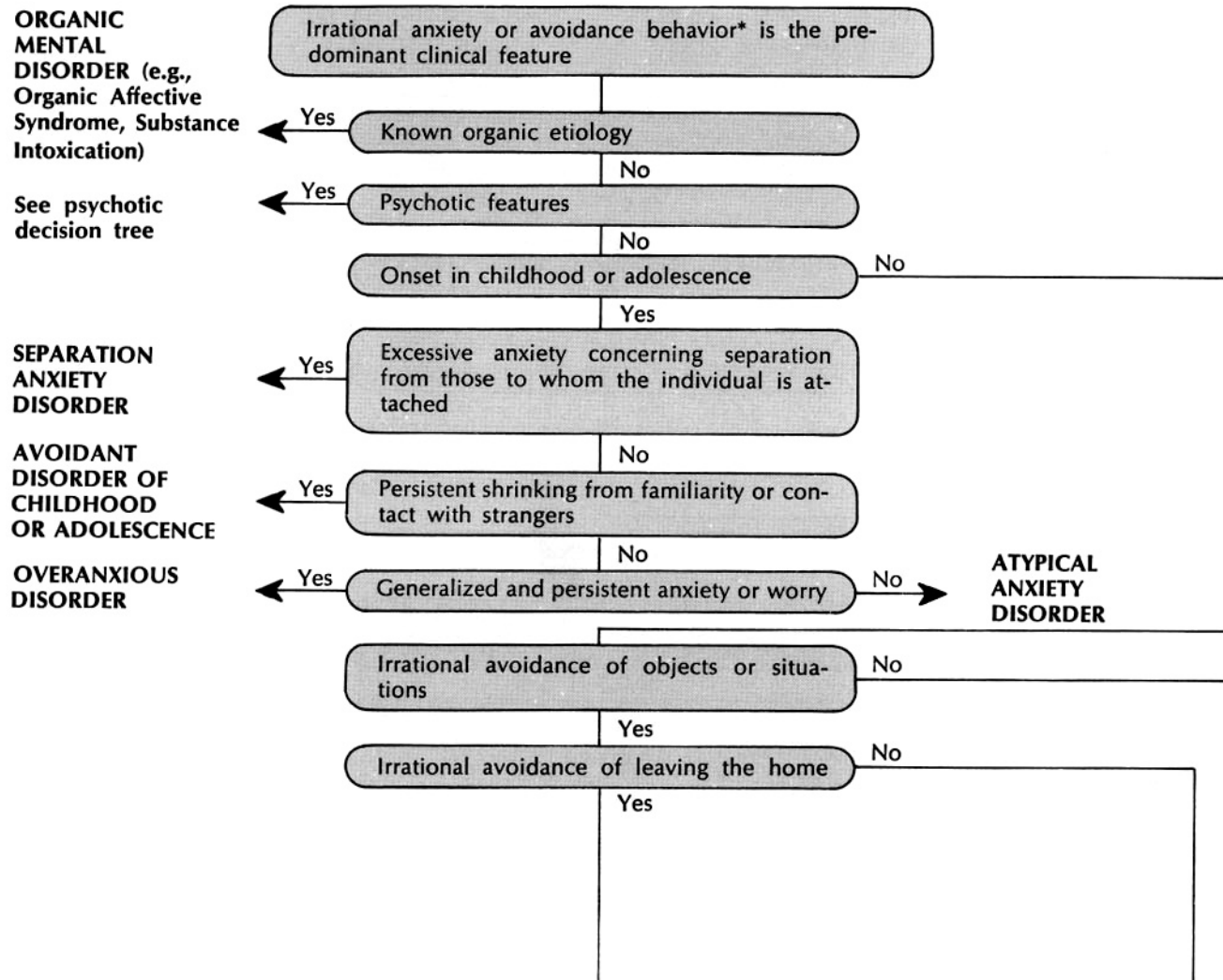
* Prepared by Robert L. Spitzer, M.D. and Janet B.W. Williams, M.S.W.

DIFFERENTIAL DIAGNOSIS OF PSYCHOTIC FEATURES





DIFFERENTIAL DIAGNOSIS OF IRRATIONAL ANXIETY AND AVOIDANCE BEHAVIOR



**AGORAPHOBIA
WITH PANIC
ATTACKS**

Yes

History of panic attacks

No

**AGORAPHOBIA
WITHOUT
PANIC ATTACKS**

**SOCIAL
PHOBIA**

Yes

Irrational avoidance of social situations
with overconcern about humiliation or em-
barrassment

No

**SIMPLE
PHOBIA**

**PANIC
DISORDER**

Yes

Recurrent panic attacks

No

**OBSESSIVE
COMPULSIVE
DISORDER**

Yes

Obsessions or compulsions

No

**GENERALIZED
ANXIETY
DISORDER**

Yes

Generalized anxiety with motor tension,
autonomic hyperactivity, apprehensive ex-
pectation, vigilance or scanning, not reac-
tive to a psychosocial stressor

No

Maladaptive reaction to a psychosocial
stressor

No

**ATYPICAL
ANXIETY
DISORDER**

**POST-TRAUMATIC
STRESS
DISORDER**

Yes

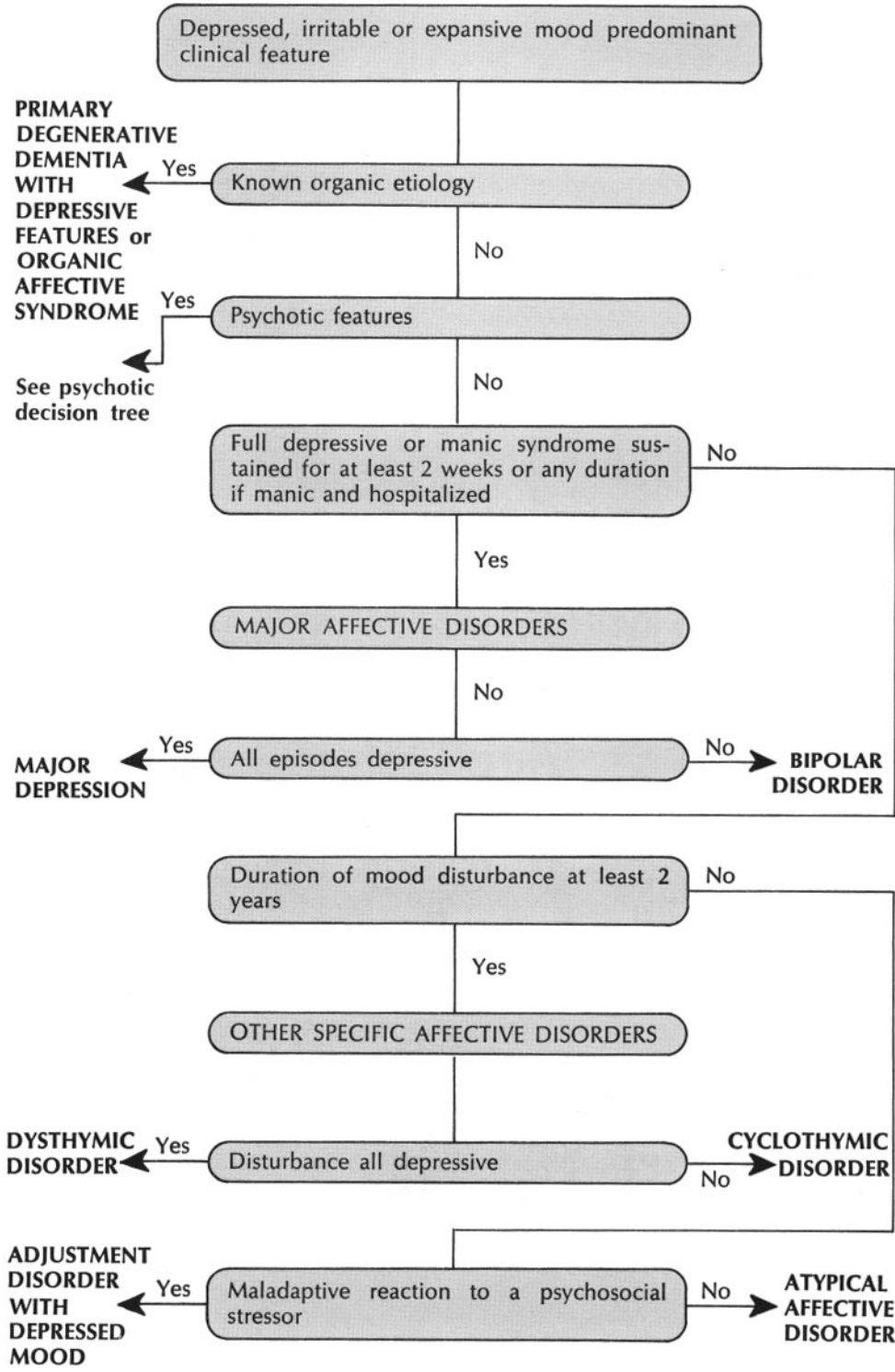
Repeated reexperiencing of traumatic
events

No

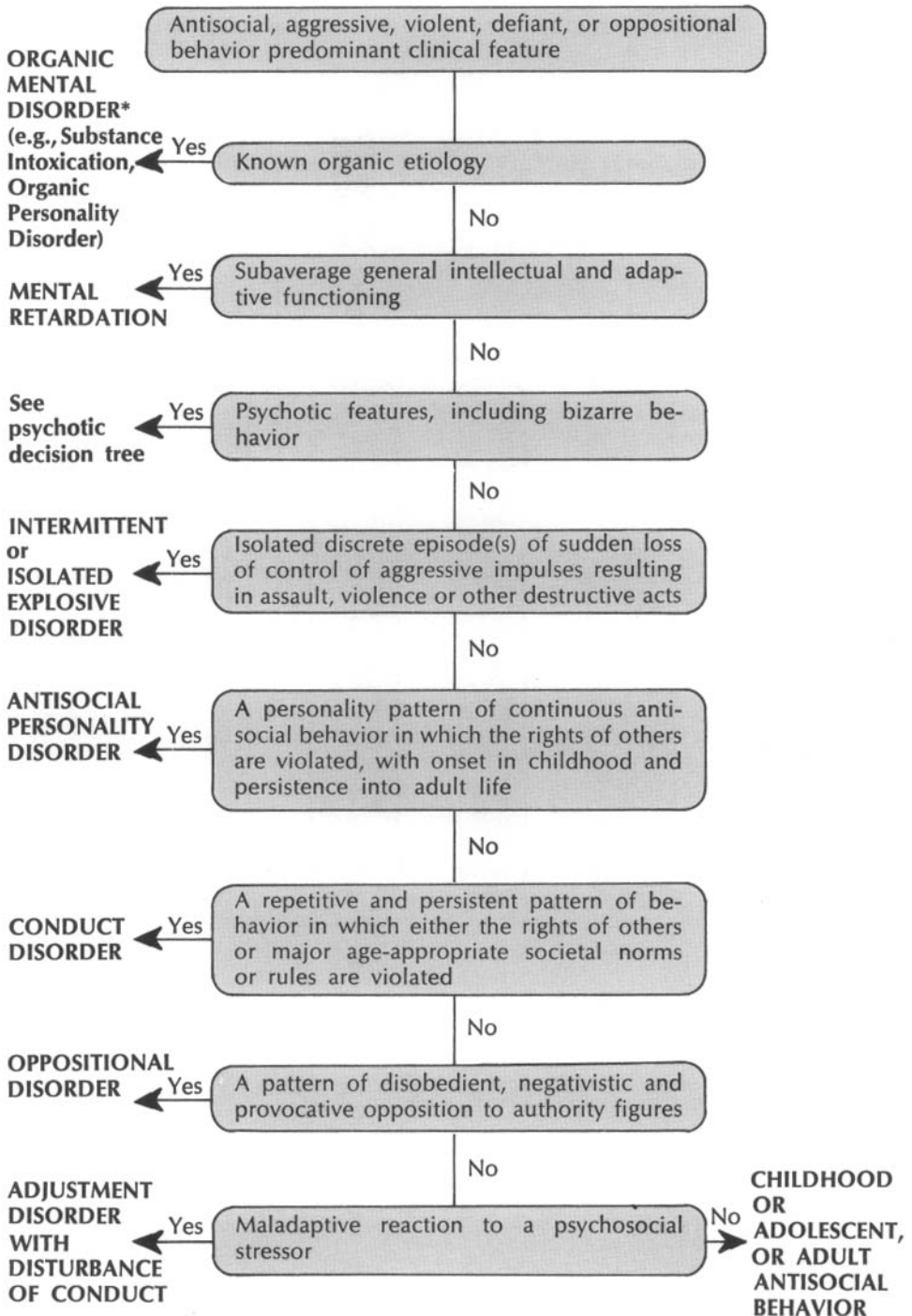
**ADJUSTMENT
DISORDER
WITH ANXIOUS
MOOD**

* Also consider Personality Disorders (Axis II), such as Avoidant, Borderline, Compulsive, and Schizotypal Personality Disorders.

DIFFERENTIAL DIAGNOSIS OF MOOD DISTURBANCE

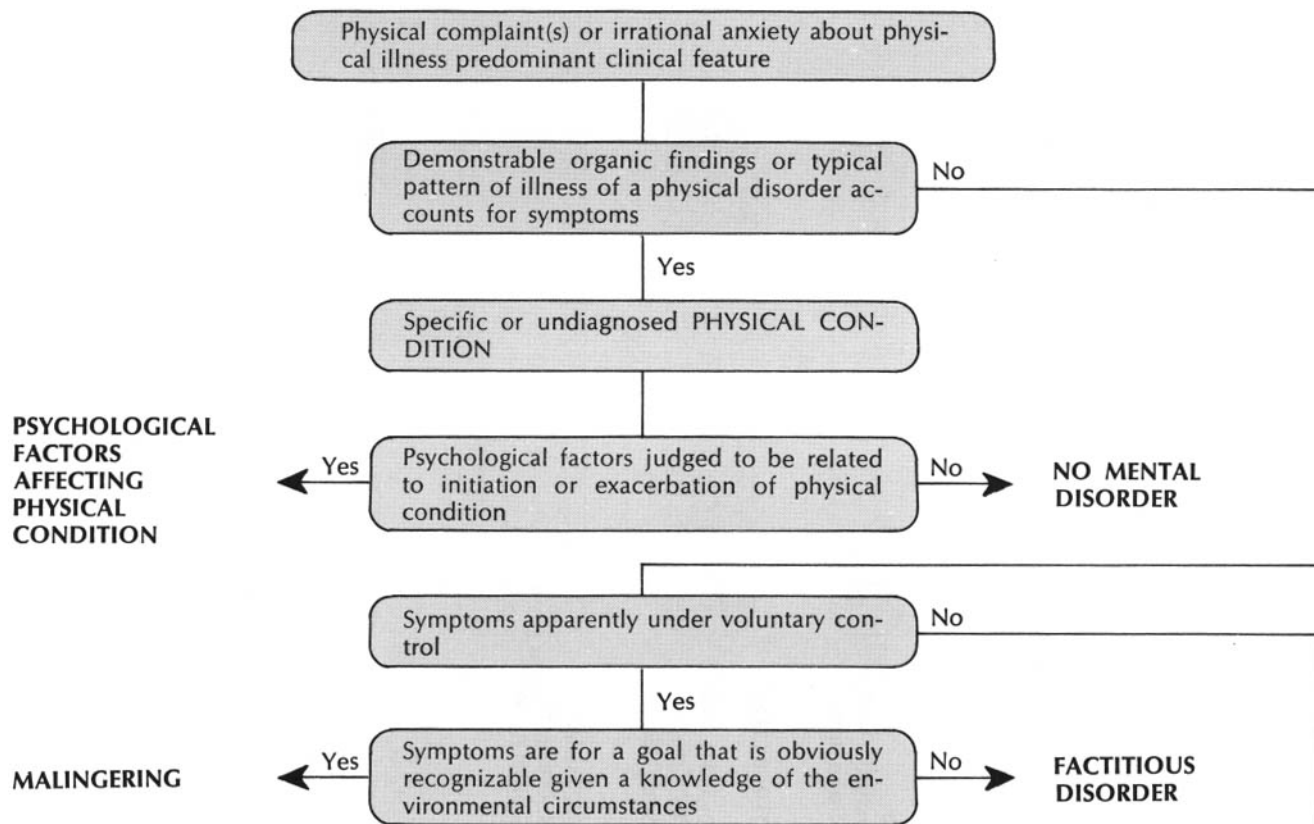


DIFFERENTIAL DIAGNOSIS OF ANTISOCIAL, AGGRESSIVE, DEFiant, OR OPPOSITIONAL BEHAVIOR



* Also consider Intermittent Explosive Disorder which can be diagnosed when symptomatic of an Organic Mental Disorder.

DIFFERENTIAL DIAGNOSIS OF PHYSICAL COMPLAINTS AND IRRATIONAL ANXIETY ABOUT PHYSICAL ILLNESS



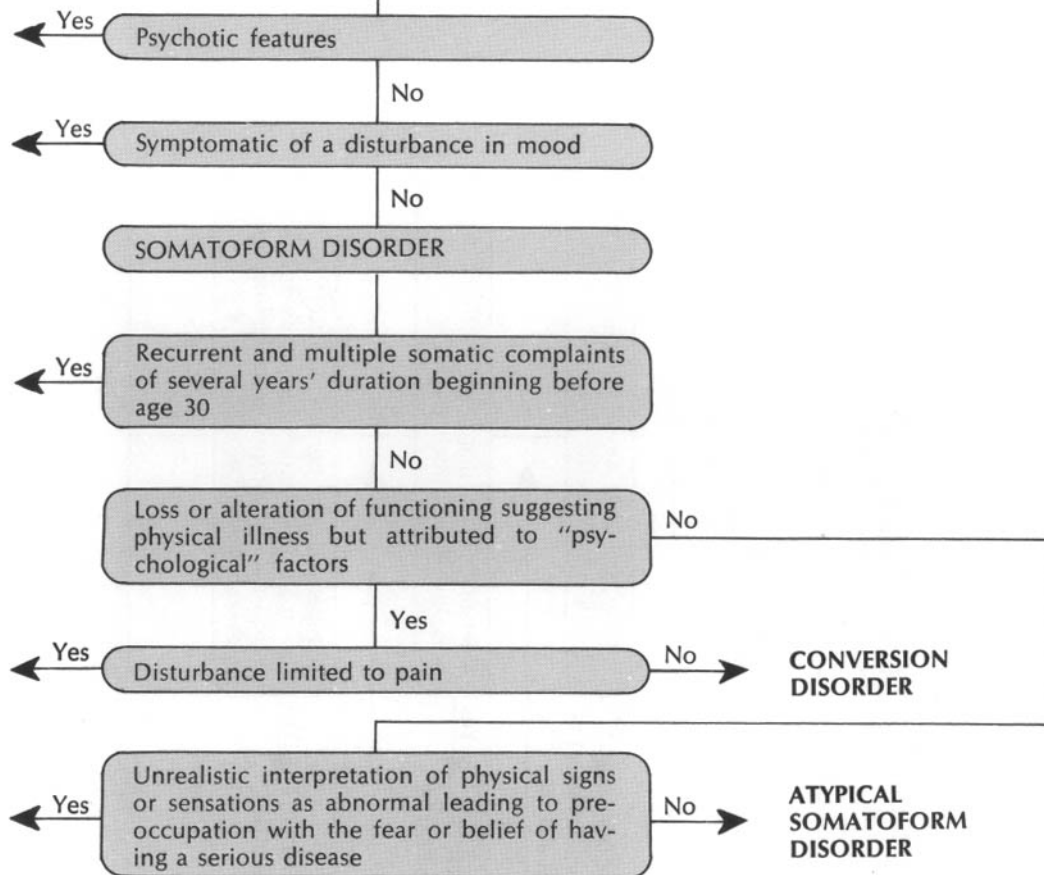
See psychotic
decision
tree

See mood
decision tree

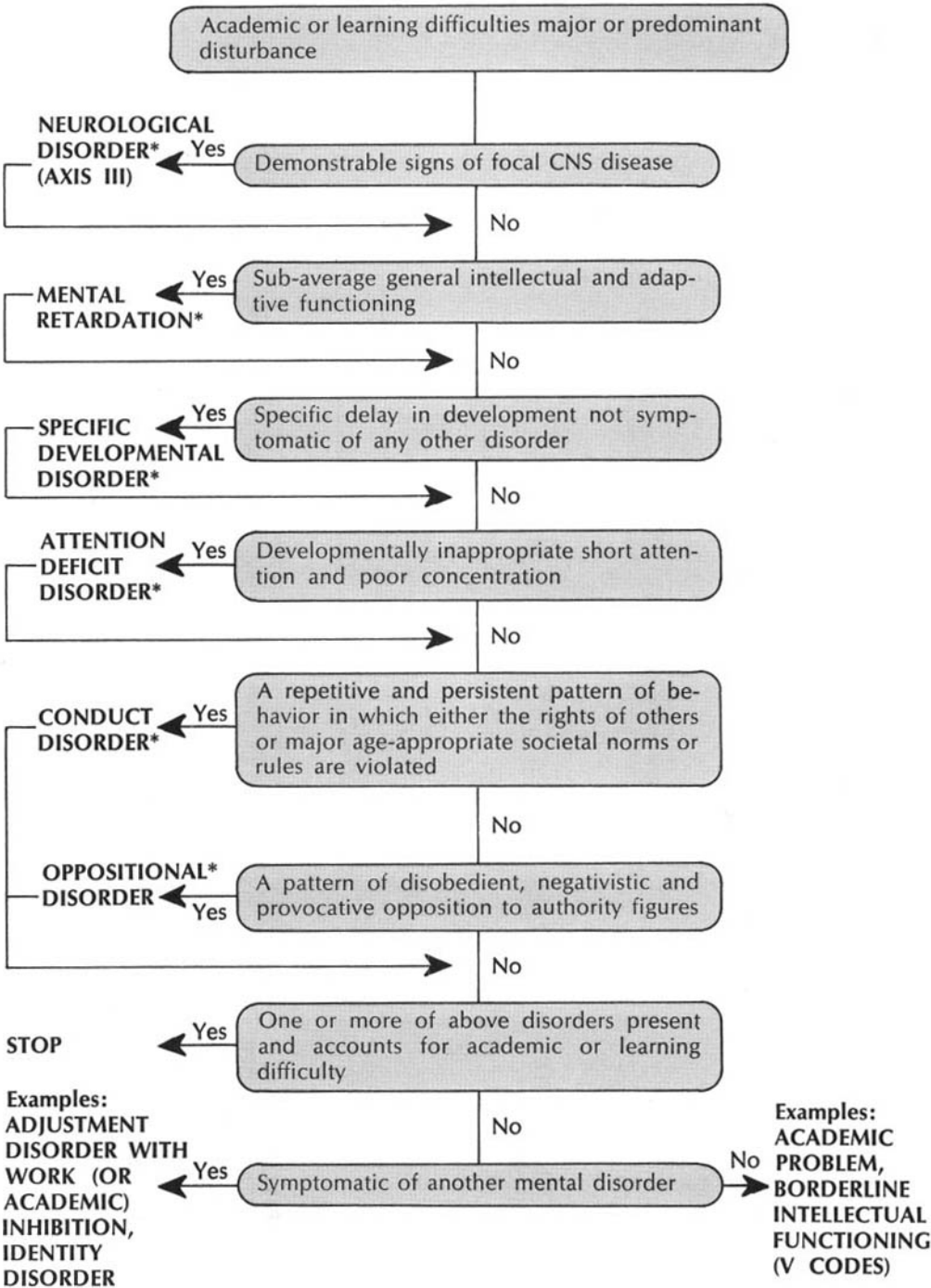
SOMATIZATION
DISORDER

PSYCHOGENIC
PAIN DISORDER

HYPO-
CHONDRIASIS

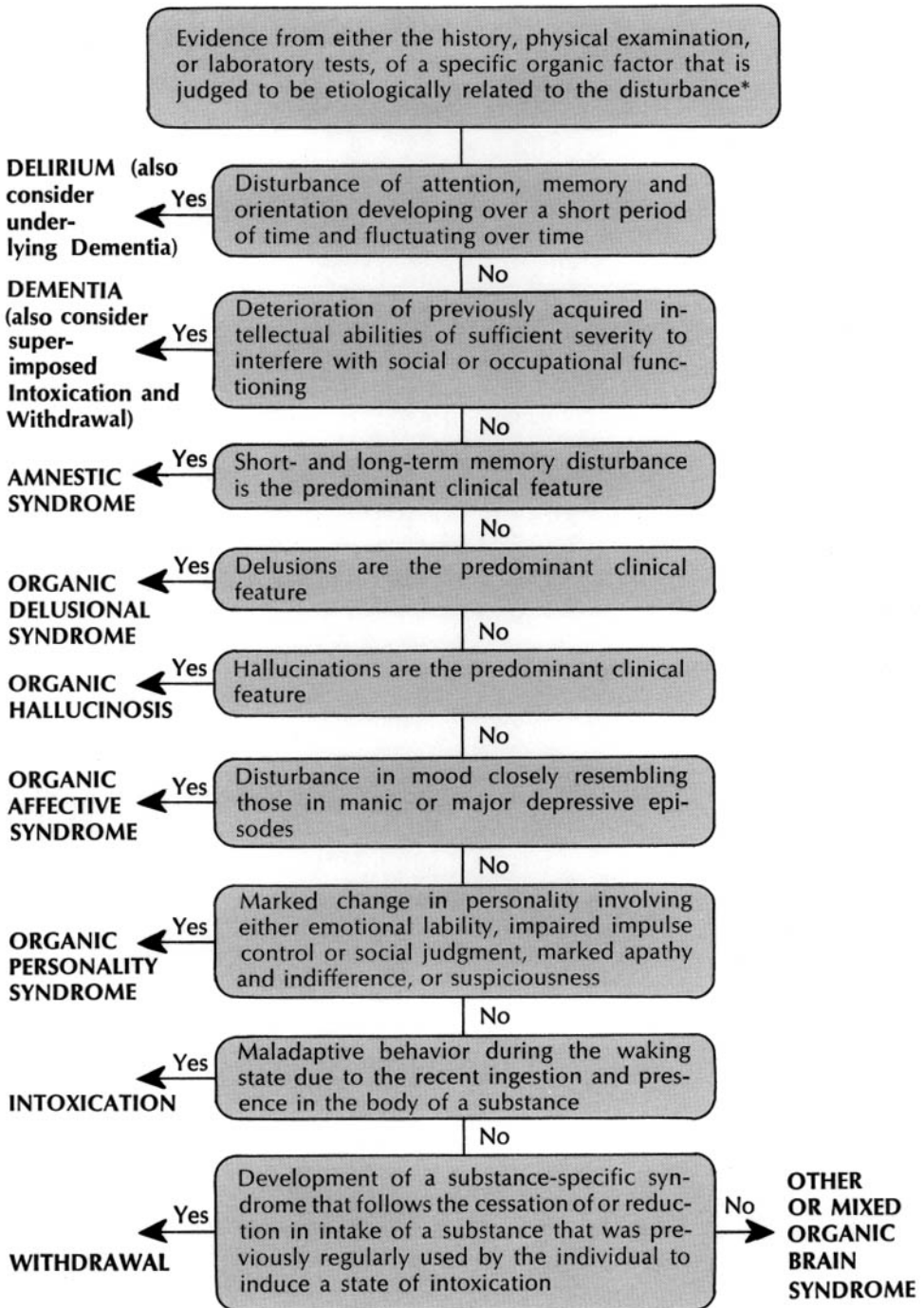


DIFFERENTIAL DIAGNOSIS OF ACADEMIC OR LEARNING DIFFICULTIES



* The arrows returning to the trunk of the tree indicate the possibility of multiple diagnoses.

DIFFERENTIAL DIAGNOSIS OF ORGANIC BRAIN SYNDROMES



* In the absence of such evidence, an organic factor can be presumed if conditions outside of the Organic Mental Disorders category have been reasonably excluded and if the disturbance meets the symptomatic criteria for Dementia.

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Glossary of Technical Terms

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Glossary of Technical Terms

DSM-III, by and large, includes only those technical terms that are essential for differential diagnosis; thus, many important terms relevant to psychopathology are omitted. This glossary of technical terms is limited to those that are used in either the descriptions and definitions or the diagnostic criteria of DSM-III and that are associated with a number of mental disorders. For example, bulimia is not in this glossary because it is rarely encountered except among Eating Disorders, and the reader wishing to learn about this symptom should refer to the text on those disorders. On the other hand, such symptoms as delusions, phobias, obsessions, and depersonalization are included because they all occur in a number of mental disorders.

Many of the entries list the disorders in which the symptom most frequently occurs; it should be understood, however, that the symptom may also be present in other disorders.

AFFECT. An immediately expressed and observed emotion. A feeling state becomes an affect when it is observable, for example, as overall demeanor or tone and modulation of voice. Affect is to be distinguished from mood, which refers to a pervasive and sustained emotion. Affect is to mood as weather is to climate. Common examples of affect are euphoria, anger, and sadness.

A range of affect may be described as *broad* (normal), *restricted* (constricted), *blunted*, or *flat*. What is considered the normal range of the expression of affect varies considerably, both within and among different cultures. The normal expression of affect involves variability in facial expression, pitch of voice, and the use of hand and body movements. Restricted affect is characterized by a clear reduction in the expressive range and intensity of affects. Blunted affect is marked by a severe reduction in the intensity of affective expression. In flat affect there is a lack of signs of affective expression; the voice may be monotonous and the face, immobile.

Affect is *inappropriate* when it is clearly discordant with the content of the person's speech or ideation. Example: A patient smiled and laughed while discussing demons who were persecuting him. An affect should not be termed inappropriate, however, when it is inappropriate merely to the situation, such as laughing when told that a relative has died; in such instances it would be more apt to refer to inappropriate behavior.

Affect is *labile* when it is characterized by repeated, rapid, and abrupt shifts. Examples: An elderly man is tearful one moment and combative the next; a young woman is observed by her friends to be friendly, gregarious, and happy one moment and angry and abusive the next, without readily apparent reason.

AGITATION. See psychomotor agitation.

ANXIETY. Apprehension, tension, or uneasiness that stems from the anticipation of danger, which may be internal or external. Some definitions of anxiety distinguish it from fear by limiting it to the anticipation of danger, the source of which is largely unknown, whereas fear is the response to a consciously recognized and usually external threat or danger. The manifestations of anxiety and fear are the same and include motor tension, autonomic hyperactivity, apprehensive expectation, and vigilance and scanning.

Anxiety may be focused on an object, situation, or activity which is avoided (phobia), or may be unfocused (free-floating anxiety). It may be experienced in discrete periods of sudden onset and be accompanied by physical symptoms (panic attacks). When anxiety is focused on physical signs or symptoms and causes preoccupation with the fear or belief of having a disease, it is termed hypochondriasis.

ATTENTION. The ability to focus in a sustained manner on one task or activity. A disturbance in attention may be manifested by difficulty in finishing tasks that have been started, easy distractibility, and/or difficulty in concentrating on work.

BLOCKING. Interruption of a train of speech before a thought or idea has been completed. After a period of silence, which may last from a few seconds to minutes, the person indicates that he or she cannot recall what he or she has been saying or meant to say. Blocking should be judged to be present only if the person spontaneously describes losing his or her thought or if upon questioning by the interviewer, the person gives that as the reason for pausing.

CATATONIC BEHAVIOR. Marked motor anomalies, generally limited to disturbances in the context of a diagnosis of a non-organic psychotic disorder.

Catatonic excitement. Excited motor activity, apparently purposeless and not influenced by external stimuli.

Catatonic negativism. An apparently motiveless resistance to all instructions or attempts to be moved. When passive, the person may resist any effort to be moved; when active, he or she may do the opposite of what is asked—for example, firmly clench jaws when asked to open mouth.

Catatonic rigidity. Maintenance of a rigid posture against all efforts to be moved.

Catatonic posturing. Voluntary assumption of an inappropriate or bizarre posture, usually held for a long period of time. Example: A patient may stand with arms outstretched as if he were Jesus on the cross.

Catatonic stupor. Marked decrease in reactivity to environment and

reduction in spontaneous movements and activity, sometimes to the point of appearing to be unaware of one's surroundings.

Catatonic waxy flexibility. The person's limbs can be "molded" into any position, which is then maintained. When the limb is being moved, it feels to the examiner as if it were made of pliable wax.

CIRCUMSTANTIALITY. A term used to describe speech that is indirect and delayed in reaching the point because of unnecessary, tedious details, and parenthetical remarks. Circumstantial replies or statements may be prolonged for many minutes if the speaker is not interrupted and urged to get to the point. Interviewers often respond to circumstantiality by interrupting the speaker in order to complete the process of history taking within an allotted time. This may make it difficult to distinguish loosening of associations from circumstantiality. In the former there is a lack of connection between clauses, but in the latter the clauses always retain a meaningful connection. In loosening of associations the original point is lost, whereas in circumstantiality the speaker is always aware of the original point, goal, or topic.

Circumstantiality is common in Compulsive Personality Disorder and in many people without mental disorder.

CLANGING. Speech in which sounds, rather than meaningful, conceptual relationships govern word choice; it may include rhyming and punning. The term is usually applied only when it is a manifestation of a pathological condition; thus, it would not be used to describe the rhyming word play of children. Example: "I'm not trying to make noise. I'm trying to make sense. If you can make sense out of nonsense, well, have fun. I'm trying to make sense out of sense. I'm not making sense (cents) anymore. I have to make dollars."

Clanging is observed most commonly in Schizophrenia and manic episodes.

COMPULSION. Repetitive and seemingly purposeful behavior that is performed according to certain rules or in a stereotyped fashion. The behavior is not an end in itself, but is designed to produce or prevent some future state of affairs; the activity, however, either is not connected in a realistic way with the state of affairs it is designed to produce or prevent, or it may be clearly excessive. The act is performed with a sense of subjective compulsion coupled with a desire to resist it (at least initially); performing the particular act is not pleasurable, although it may afford some relief of tension. Example: An individual feels compelled to wash his or her hands, every time he or she shakes hands because of a fear of contamination that he or she recognizes as excessive.

Compulsions are characteristic of Obsessive Compulsive Disorder and may also be seen in Schizophrenia.

CONFABULATION. Fabrication of facts or events in response to questions about situations or events that are not recalled because of memory impairment. It differs from lying in that the individual is not consciously attempting to deceive.

Confabulation is common in Organic Amnestic Syndrome.

CONVERSION SYMPTOM. A loss or alteration of physical functioning that suggests a physical disorder but that is actually a direct expression of a psychological conflict or need. The disturbance is not under voluntary control, and is not explained by any physical disorder (this possibility having been excluded by appropriate investigation).

Conversion symptoms are observed in Conversion Disorder, and may occur in Schizophrenia.

DELUSION. A false personal belief based on incorrect inference about external reality and firmly sustained in spite of what almost everyone else believes and in spite of what constitutes incontrovertible and obvious proof or evidence to the contrary. The belief is not one ordinarily accepted by other members of the person's culture or subculture (i.e., it is not an article of religious faith).

When a false belief involves an extreme value judgment, it is regarded as a delusion only when the judgment is so extreme as to defy credibility. Example: If someone claims he or she is terrible and has disappointed his or her family, this is generally not regarded as a delusion even if an objective assessment of the situation would lead observers to think otherwise; but if someone claims he or she is the worst sinner in the world, this would generally be considered a delusional conviction. Similarly, a person judged by most people to be moderately underweight who asserted he or she was fat would not be regarded as delusional; but one with Anorexia Nervosa who at the point of extreme emaciation insisted he or she was fat could rightly be considered delusional.

A delusion should be distinguished from a hallucination, which is a false sensory perception (although a hallucination may give rise to the delusion that the perception is true). A delusion is also to be distinguished from an overvalued idea, in which an unreasonable belief or idea is not as firmly held as is the case with a delusion.

Delusions are subdivided according to their content. Some of the more common types are listed below.

Delusion of being controlled. A delusion in which feelings, impulses, thoughts, or actions are experienced as being not one's own, as being imposed by some external force. This does not include the mere conviction that one is acting as an agent of God, has had a curse placed on him or her, is the victim of fate, or is not sufficiently assertive. The symptom should be judged present only when the subject experiences his or her will, thoughts, or feelings as operating under some external force. Examples: A man claimed that his words were not his own, but those of his father; a student believed that his actions were under the control of a yogi; a housewife believed that sexual feelings were being put into her body from without.

Delusion, bizarre. A false belief whose content is patently absurd and has no possible basis in fact. Example: A man believed that when his adenoids had been removed in childhood, a box had been inserted into his

head, and that wires had been placed in his head so that the voice he heard was that of the governor.

Delusion, grandiose. A delusion whose content involves an exaggerated sense of one's importance, power, knowledge, or identity. It may have a religious, somatic, or other theme.

Delusional jealousy. The delusion that one's sexual partner is unfaithful.

Delusion, mood-congruent. See mood-congruent psychotic features.

Delusion, mood-incongruent. See mood-incongruent psychotic features.

Delusion, nihilistic. A delusion involving the theme of non-existence of the self or part of the self, others, or the world. Examples: "The world is finished"; "I no longer have a brain"; "There is no need to eat, because I have no insides." A somatic delusion may also be a nihilistic delusion if the emphasis is on nonexistence of the body or a part of the body.

Delusion, persecutory. A delusion in which the central theme is that a person or group is being attacked, harassed, cheated, persecuted, or conspired against. Usually the subject or someone or some group or institution close to him or her is singled out as the object of the persecution.

It is recommended that the term "paranoid delusion" not be used, because its meanings are multiple, confusing, and contradictory. It has often been employed to refer to both persecutory and grandiose delusions because of their presence in the paranoid subtype of Schizophrenia.

Delusion of poverty. A delusion that the person is, or will be, bereft of all, or virtually all, material possessions.

Delusion of reference. A delusion whose theme is that events, objects, or other people in the person's immediate environment have a particular and unusual significance, usually of a negative or pejorative nature. This differs from an idea of reference, in which the false belief is not as firmly held as in a delusion. If the delusion of reference involves a persecutory theme, then a delusion of persecution is present as well. Examples: A woman was convinced that programs on the radio were directed especially to her; when recipes were broadcast, it was to tell her to prepare wholesome food for her child and stop feeding her candy; when dance music was broadcast, it was to tell her to stop what she was doing and start dancing, and perhaps even to resume ballet lessons. A patient notes that the room number of his therapist's office is the same as the number of the hospital room in which his father died and feels that this means there is a plot to kill him.

Delusion, somatic. A delusion whose main content pertains to the functioning of one's body. Examples: One's brain is rotting; one is pregnant despite being postmenopausal.

Extreme value judgments about the body may, under certain circumstances, also be considered somatic delusions. Example: A person insists that his nose is grossly misshaped despite lack of confirmation of this by observers.

Hypochondriacal delusions are also somatic delusions when they involve specific changes in the functioning or structure of the body rather than merely an insistent belief that one has a disease.

Delusions, systematized. A single delusion with multiple elaborations or a group of delusions that are all related by the individual to a single event or theme. Example: A man who failed his bar examination developed the delusion that this occurred because of a conspiracy involving the university and the bar association. He then attributed all other difficulties in his social and occupational life to this continuing conspiracy.

DEPERSONALIZATION. An alteration in the perception or experience of the self so that the feeling of one's own reality is temporarily lost. This is manifested in a sense of self-estrangement or unreality, which may include the feeling that one's extremities have changed in size, or a sense of seeming to perceive oneself from a distance (usually from above).

Depersonalization is seen in Depersonalization Disorder, and may also occur in Schizotypal Personality Disorder, and Schizophrenia. It also occurs in the absence of any mental disorder in the presence of overwhelming anxiety, stress or fatigue.

DIAGNOSIS. In general usage, and in DSM-III, this term refers to the process of identifying specific mental or physical disorders. Some, however, use the term more broadly to refer to a comprehensive evaluation that is not limited to the identification of specific disorders. Thus in the limited sense of the term, only Axes I, II and III are diagnostic, whereas in the broader sense, Axes IV and V and other aspects of assessment can also be considered diagnostic.

DISORIENTATION. Confusion about the date or time of day, where one is (place), or who one is (identity).

Disorientation is characteristic of some Organic Mental Disorders, such as Delirium and Dementia.

DISTRACTIBILITY. Attention drawn too frequently to unimportant or irrelevant external stimuli. Example: While being interviewed, a subject's attention is repeatedly drawn to noise from an adjoining office, a book that is on a shelf, or the interviewer's school ring.

ECHOLALIA. Repetition (echoing) of the words or phrases of others. Typical

echolalia tends to be repetitive and persistent. The echo is often uttered with a mocking, mumbling, or staccato intonation. Echolalia should not be confused with habitual repetition of questions, apparently to clarify the question and formulate its answer, as when a patient is asked, "When did you come to the hospital?" and replies, "Come to the hospital? Yesterday."

Example: An interviewer says to a subject, "I'd like to talk with you for a few minutes"; and the subject responds, in a mumbling tone, "Talk with you for a few minutes. Talk with you for a few minutes."

Echolalia is observed in some Pervasive Developmental Disorders, Organic Mental Disorders, and in Schizophrenia.

EGO-DYSTONIC. A symptom or personality trait that is recognized by the individual as unacceptable and undesirable and is experienced as alien. Examples: obsessions and compulsions; a homosexual arousal pattern that is unacceptable to the individual would be ego-dystonic, whereas, if the individual were not distressed by the pattern and experienced it as acceptable, it would be ego-syntonic.

FLIGHT OF IDEAS. A nearly continuous flow of accelerated speech with abrupt changes from topic to topic, usually based on understandable associations, distracting stimuli, or plays on words. When severe, the speech may be disorganized and incoherent.

Flight of ideas is most frequently seen in manic episodes, but may also be observed in some cases of Organic Mental Disorders, Schizophrenia, other psychotic disorders, and, occasionally, acute reactions to stress.

FORMAL THOUGHT DISORDER. A disturbance in the form of thought as distinguished from the content of thought. The boundaries of the concept are not clear and there is no consensus as to which disturbances in speech or thought are included in the concept. For this reason, "formal thought disorder" is not used as a specific descriptive term in DSM-III. See loosening of associations, incoherence, poverty of content of speech, neologisms, perseveration, blocking, echolalia, clanging.

GRANDIOSITY. An inflated appraisal of one's worth, power, knowledge, importance, or identity. When extreme, grandiosity may be of delusional proportions. Example: A professor who frequently puts his students to sleep with his boring lectures is convinced that he is one of the more dynamic and exciting teachers at the university.

HALLUCINATION. A sensory perception without external stimulation of the relevant sensory organ. A hallucination has the immediate sense of reality of a true perception, although in some instances the source of the hallucination may be perceived as within the body (e.g., an auditory hallucination may be experienced as coming from within the head rather than through the ears). (Some investigators limit the concept of true hallucinations to sensations whose source

is perceived as being external to the body, but the clinical significance of this distinction has yet to be demonstrated, so it is not made in this manual.)

There may or may not be a delusional interpretation of the hallucinatory experience. For example, one person with auditory hallucinations may recognize that he or she is having a false sensory experience whereas another may be convinced that the source of the sensory experience has an independent physical reality. Strictly speaking, hallucinations indicate a psychotic disturbance only when they are associated with gross impairment in reality testing (see psychotic). The term hallucination, by itself, is not ordinarily applied to the false perceptions that occur during dreaming, while falling asleep (hypnagogic), or when awakening (hypnopompic). Hallucinations occurring in the course of an intensely shared religious experience generally have no pathological significance.

Hallucinations should be distinguished from illusions, in which an external stimulus is misperceived or misinterpreted, and from normal thought processes that are exceptionally vivid. Transient hallucinatory experiences are common in individuals without mental disorder.

Hallucination, auditory. A hallucination of sound, most commonly of voices, but sometimes of clicks, rushing noises, music, etc.

Hallucination, gustatory. A hallucination of taste, unpleasant tastes being the most common.

Hallucination, mood-congruent. See mood-congruent psychotic features.

Hallucination, mood-incongruent. See mood-incongruent psychotic features.

Hallucination, olfactory. A hallucination involving smell. Example: A woman complained of a persistent smell of dead bodies. Some individuals are convinced they have a body odor they themselves cannot smell; this symptom is a delusion, not an olfactory hallucination.

Hallucination, somatic. A hallucination involving the perception of a physical experience localized within the body. Example: A feeling of electricity running through one's body.

Somatic hallucinations are to be distinguished from unexplained physical sensations; a somatic hallucination can be identified with certainty only when a delusional interpretation of a physical illness is present. A somatic hallucination is to be distinguished also from hypochondriacal preoccupation with, or exaggeration of, normal physical sensations and from a tactile hallucination, in which the sensation is usually related to the skin.

Hallucination, tactile. A hallucination involving the sense of touch, often of something on or under the skin. Almost invariably the symptom is associated with a delusional interpretation of the sensation. Examples:

A man said he could feel the Devil putting pins into his flesh; another claimed he could feel himself being penetrated anally; still another complained of experiencing pains, which he attributed to the Devil, throughout his body, although there was no evidence of any physical illness.

A particular tactile hallucination is *formication*, which is the sensation of something creeping or crawling on or under the skin. Often there is a delusional interpretation of the sensation, as when it is attributed to insects or worms. Formication is seen in Alcohol Withdrawal Delirium and the withdrawal phase of Cocaine Intoxication.

Tactile hallucinations of pain are to be distinguished from Psychogenic Pain Disorder, in which there is no delusional interpretation.

Hallucination, visual. A hallucination involving sight, which may consist of formed images, such as of people, or of unformed images, such as flashes of light. Visual hallucinations should be distinguished from illusions, which are misperceptions of real external stimuli.

HYPOCHONDRIASIS. Unrealistic interpretation of physical signs or sensations as abnormal, leading to preoccupation with the fear or belief of having a disease.

IDEA OF REFERENCE. An idea, held less firmly than a delusion, that events, objects, or other people in the person's immediate environment have a particular and unusual meaning specifically for him or her. See also delusion of reference.

IDENTITY. The sense of self, providing a unity of personality over time. Prominent disturbances in identity or the sense of self are seen in Schizophrenia, Borderline Personality Disorder, and Identity Disorder.

ILLOGICAL THINKING. Thinking that contains clear internal contradictions or in which conclusions are reached that are clearly erroneous, given the initial premises. It may be seen in individuals without mental disorder, particularly in situations in which they are distracted or fatigued. Illogical thinking has psychopathological significance only when it is marked, as in the examples noted below, and when it is not due to cultural or religious values or to intellectual deficit. Markedly illogical thinking may lead to, or result from, a delusional belief or may be observed in the absence of a delusion.

Examples: A patient explained that she gave her family IBM cards, which she punched, in an effort to improve communication with them. Another patient stated: "Parents are the people that raise you. Parents can be anything—material, vegetable, or mineral—that has taught you something. A person can look at a rock and learn something from it, so a rock is a parent." In response to the question "Why did you go to Kingston?" a patient replied, "Because I believe in the King James Bible and my name is James. I went to Kingston to see the Queen."

ILLUSION. A misperception of a real external stimulus. Examples: The rustling of leaves is heard as the sound of voices; a man claims that when he looks in a mirror, he sees his face distorted and misshapen. See also hallucination.

INCOHERENCE. Speech that, for the most part, is not understandable, owing to any of the following: a lack of logical or meaningful connection between words, phrases, or sentences; excessive use of incomplete sentences; excessive irrelevancies or abrupt changes in subject matter; idiosyncratic word usage; distorted grammar. Mildly ungrammatical constructions or idiomatic usages characteristic of particular regional or ethnic backgrounds, lack of education, or low intelligence should not be considered incoherence; and the term is generally not applied when there is evidence that the disturbance in speech is due to an aphasia.

Example: Interviewer: "Why do you think people believe in God?" Subject: "Um, because making a do in life. Isn't none of that stuff about evolution guiding isn't true anymore now. It all happened a long time ago. It happened in eons and eons and stuff they wouldn't believe in Him. The time that Jesus Christ people believe in their thing people believed in, Jehovah God that they didn't believe in Jesus Christ that much."

Incoherence may be seen in some Organic Mental Disorders, Schizophrenia, and other psychotic disorders.

INSOMNIA. Difficulty falling or staying asleep. Initial insomnia is difficulty in falling asleep. Middle insomnia involves an awakening, followed by difficulty returning to sleep, but eventually doing so. Terminal insomnia is awakening at least two hours before one's usual waking time and being unable to return to sleep.

LOOSENING OF ASSOCIATIONS. Thinking characterized by speech in which ideas shift from one subject to another that is completely unrelated or only obliquely related without the speaker's showing any awareness that the topics are unconnected. Statements that lack a meaningful relationship may be juxtaposed, or the individual may shift idiosyncratically from one frame of reference to another: When loosening of associations is severe, speech may be incoherent. The term is generally not applied when abrupt shifts in topics are associated with a nearly continuous flow of accelerated speech (as in flight of ideas).

Example: Interviewer: "What did you think of the whole Watergate affair?" Subject: "You know I didn't tune in on that, I felt so bad about it. I said, Boy, I'm not going to know what's going on in this. But it seemed to get so murky, and everybody's reports were so negative. Huh, I thought, I don't want any part of this, and I don't care who was in on it, and all I could figure out was Artie had something to do with it. Artie was trying to flush the bathroom toilet of the White House or something. She was trying to do something fairly simple. The tour guests stuck or something. She got blamed because the water overflowed, went down in the basement, down, to the kitchen. They had a, they were going to have to repaint and restore the White House room, the enormous living room. And then it was at this reunion they were having. And it's just such a mess and I just thought, well, I'm just going to pretend like I don't even know what's going on. So I came downstairs and 'cause I pretended like I didn't know what was going on, I slipped on the floor of the

kitchen, cracking my toe, when I was teaching some kids how to do some double dives."

Loosening of associations may be seen in Schizophrenia, manic episodes, and other psychotic disorders.

MAGICAL THINKING. The individual believes that his or her thoughts, words, or actions might, or will in some manner cause or prevent a specific outcome in some way that defies the normal laws of cause and effect. Example: A man believed that if he said a specific prayer three times each night, his mother's death might be prevented indefinitely; a mother believed that if she had an angry thought her child would become ill.

Magical thinking may be part of ideas of reference or may reach delusional proportions when the individual maintains a firm conviction about the belief despite evidence to the contrary.

Magical thinking is seen in children, in people in primitive cultures, and in Schizotypal Personality Disorder, Schizophrenia, and Obsessive Compulsive Disorder.

MENTAL DISORDER. In DSM-III, a mental disorder is conceptualized as a clinically significant behavioral or psychologic syndrome or pattern that occurs in an individual and that typically is associated with either a painful symptom (distress) or impairment in one or more important areas of functioning (disability). In addition, there is an inference that there is a behavioral, psychologic, or biologic dysfunction, and that the disturbance is not only in the relationship between the individual and society. When the disturbance is limited to a conflict between an individual and society, this may represent social deviance, which may or may not be commendable, but is not by itself a mental disorder.

MOOD. A pervasive and sustained emotion that in the extreme, markedly colors the person's perception of the world. Mood is to affect as climate is to weather. Common examples of mood include depression, elation, anger, and anxiety.

Mood, dysphoric. An unpleasant mood, such as depression, anxiety, or irritability.

Mood, elevated. A mood that is more cheerful than normal; it does not necessarily imply pathology.

Mood, euphoric. An exaggerated feeling of well-being. As a technical term, euphoria implies a pathological mood. Whereas the individual with a normally elevated mood may describe himself or herself as being in "good spirits," "very happy," or "cheerful," the euphoric person is likely to exclaim that he or she is "on top of the world," "up in the clouds," or to say, "I feel ecstatic," "I'm flying," or "I am high."

Mood, euthymic. Mood in the "normal" range, which implies the absence of depressed or elevated mood.

Mood, expansive. Lack of restraint in expressing one's feelings, frequently with an overvaluation of one's significance or importance. There may also be elevated or euphoric mood.

Mood, irritable. Internalized feeling of tension associated with being easily annoyed and provoked to anger.

MOOD-CONGRUENT PSYCHOTIC FEATURES. Delusions or hallucinations whose content is entirely consistent with either a depressed or a manic mood. If the mood is depressed, the content of the delusions or hallucinations would involve themes of either personal inadequacy, guilt, disease, death, nihilism, or deserved punishment. If the mood is manic, the content of the delusions or hallucinations would involve themes of inflated worth, power, knowledge, or identity, or special relationship to a deity or a famous person.

MOOD-INCONGRUENT PSYCHOTIC FEATURES. Delusions or hallucinations whose content is not consistent with either a depressed or a manic mood. In the case of depression, a delusion or hallucination whose content does not involve themes of either personal inadequacy, guilt, disease, death, nihilism, or deserved punishment. In the case of mania, a delusion or hallucination whose content does not involve themes of either inflated worth, power, knowledge, or identity or special relationship with a deity or a famous person. Examples of such symptoms are persecutory delusions, thought insertion, thought broadcasting, and delusions of being controlled, whose content has no apparent relationship to any of the themes noted above. (Note: The catatonic symptoms of stupor, mutism, negativism, and posturing in manic episodes are also considered mood-incongruent psychotic features.)

NEOLOGISMS. New words invented by the subject, distortions of words, or standard words to which the subject has given new, highly idiosyncratic meaning. The judgment that neologisms are present should be made cautiously, taking into account the subject's educational and cultural background. Examples: "I was accused of midigation" (meaning the subject was accused of breaking the law). "They had an insinuating machine next door" (person explaining how her neighbors were bothering her).

Neologisms may be observed in Schizophrenia and other psychotic disorders.

NEUROTIC DISORDER. A mental disorder in which the predominant disturbance is a symptom or group of symptoms that is distressing to the individual and is recognized by him or her as unacceptable and alien (ego-dystonic); reality testing is grossly intact. Behavior does not actively violate gross social norms (though it may be quite disabling). The disturbance is relatively enduring or recurrent without treatment, and is not limited to a transitory reaction to stressors. There is no demonstrable organic etiology or factor.

NEUROTIC PROCESS. A specific etiological process involving the following

sequence: (1) unconscious conflicts between opposing desires or between desires and prohibitions, which cause (2) unconscious perception of anticipated danger or dysphoria, which leads to (3) use of defense mechanisms that result in (4) either symptoms, personality disturbance, or both.

OBSSESSIONS. Recurrent, persistent ideas, thoughts, images, or impulses that are ego-dystonic, that is, they are not experienced as voluntarily produced, but rather as ideas that invade consciousness.

Obsessions are characteristic of Obsessive Compulsive Disorder and may also be seen in Schizophrenia.

ORIENTATION. Awareness of where one is in relation to time, place, and person.

OVERVALUED IDEA. An unreasonable and sustained belief or idea that is maintained with less than delusional intensity. It differs from an obsessional thought in that the person holding the overvalued idea does not recognize its absurdity and thus does not struggle against it. As with a delusion, the idea or belief is not one that is ordinarily accepted by other members of the person's culture or subculture.

Example: A patient with a long-standing hand-washing compulsion thought there might be danger in shaking hands with people, because they might have recently been inoculated against smallpox and be infectious. Although she acknowledged that the danger might not be real, she could not accept reassurances that there was medically no danger.

PANIC ATTACKS. Discrete periods of sudden onset of intense apprehension, fearfulness, or terror, often associated with feelings of impending doom. During the attacks there are such symptoms as dyspnea, palpitations, chest pain or discomfort, choking or smothering sensations, and fear of going crazy or losing control.

Panic attacks are characteristic of Panic Disorder, but may also occur in Somatization Disorder, Major Depression and Schizophrenia.

PARANOID IDEATION. Ideation, of less than delusional proportions, involving suspiciousness or the belief that one is being harassed, persecuted, or unfairly treated. In some instances the term is used when the clinician is unsure of whether the disturbances are actually delusional. Ideas of reference often involve paranoid ideation.

PERSEVERATION. Persistent repetition of words, ideas, or subjects so that, once an individual begins speaking about a particular subject or uses a particular word, it continually recurs. Perseveration differs from the repetitive use of "stock words" or interjections such as "you know" or "like."

Examples: "I think I'll put on my hat, my hat, my hat, my hat." Interviewer: "Tell me what you are like, what kind of person you are." Subject: